

ICRH-K IMPACT REPORT

Jan-Dec
2021





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LIST OF ABBREVIATIONS:

ICRHK	International Centre for Reproductive Health Kenya
KP	Key Populations
TG	Transgender persons
SRH	Sexual and Reproductive Health
FSW	Female sex workers
MSM	Men having sex with men
PWID	People who inject drugs
PE	Peer Educators
ODK	Open Data kit
PrEP	Pre exposure prophylaxis
KNBS	Kenya National Bureau of Statistics
MoH	Ministry of Health
REs	Resident Enumerators



PART I:

INTRODUCTION



In Kenya **since 2000**



Working in **28 counties**



88 full-time employees



Approaches founded
on the most recent
scientific evidence



ICRHK BOARD:

Prof. Kishor Mandaliya	Board Chair
Prof. Stanley Luchters	Board Secretary
Dr. Esther Getambu	Board Treasurer
Prof. Walter Jaoko	Member
Prof. Marleen Temmerman	Member
Dr. Jennifer Othigo	Member
Dr. James Patrick Ochieng Odera	Member
Lou Dierick	Member
Nelly Munyasia	Member
Prof. Violet Naanyu	Member
Dr. S.K. Sharif	Member

ICRHK MANAGEMENT

Dr. Griffins Manguro	Country Director
Irene Kiminza	Chief Finance Officer
Mary Thiongo	Senior Technical Advisor
Dr. Patricia Owira	Senior Technical Advisor
Gerald Githinji	Monitoring & Evaluation Advisor

ABOUT ICRHK

The International Center for Reproductive Health (ICRH)-Kenya (ICRHK) was founded in 2000 as **an independent local non-governmental organization (NGO)**. Despite its independence, ICRH Kenya is a member of the ICRH Global network and is linked to ICRH offices in Belgium and Mozambique.

ICRHK employed a total of 88 full-time employees in 2021. A further 415 part-time staff were hired as Resident Enumerators in counties to collect data as part of the Performance Monitoring for Action and Third-Party Monitoring for the DESIP and In-Their-Hands Projects.

Staff are knowledgeable about conducting research as well as program design and implementation. The organization carries out large-scale community-based behavioural change programs as well as research on sexual and reproductive health and rights (SRHR). The teams are multidisciplinary, with researchers, clinical teams, social scientists, and monitoring, evaluation, and program learning experts on board. ICRHK approaches are founded on the most recent scientific evidence. The research and interventions are both embedded in ongoing conversations and feedback from the communities served.

Over the last two decades, ICRHK has worked with a variety of organizations to implement high-quality sexual and reproductive health (SRH) programs. ICRHK has developed comprehensive and efficient processes for the programmatic and financial management of donor funding as a result of this collaboration and experience.

ICRH Kenya programs are available in 28 of Kenya's 42 counties. We have projects in the counties listed below. Bungoma, Busia, Homabay, Kajiado, Kakamega, Kilifi, Kisii, Kisumu, Migori, Uasin Gishu, Mombasa, Nairobi, Nakuru, Nyamira, Siaya, Vihiga, Kitui, Nandi, Kiambu, Kericho, West Pokot, Kwale, Makueni, Machakos, Samburu, Lamu, Taita Taveta.



VISION:

The Vision of ICRHK is to be a Centre of Excellence in Sexual & Reproductive Health Research and Interventions.



THE MISSION:

To contribute to improve health status of Kenyans by designing and implementing innovative, evidence-based and cost-effective interventions and conduct research capable of influencing public policy and practice in the field of sexual and reproductive health in Kenya.



OUR APPROACH:

We design and implement innovative evidence-based and cost effective interventions and undertake research in the field of sexual and reproductive health while; engaging with stakeholders throughout our work, delivering on promises, engaging in policy dialogues rooted strictly in scientific evidence and promoting positive change by ensuring that our outputs are locally and globally relevant.

DONORS & PARTNERS:

We collaborated with several donors and partners in 2021. The following donors provided funding for ICRHK research and projects, which were carried out in collaboration with the following institutions/organizations.



DONORS:

USAID/PEPFAR, Bill & Melinda Gates Foundation, UNFPA, Kindernothilfe, Foundation for Innovation in New Diagnostics (FIND), Foreign Commonwealth & Development Office (FCDO), Children's Investment Fund Foundation (CIFF), AmplifyChange, Grand Challenges Canada.

PARTNERS:

Pathfinder International, LVCT Health (Stawisha Pwani), Hera, Aga Khan University, Jhpiego Kenya, Jhpiego Baltimore, Jilinde, the Equality Effect, Technical University of Mombasa, The Coast General Teaching & Referral Hospital.

COLLABORATING INSTITUTIONS:

Ministry of Health (MoH)
NASOP, National Council for Population and Development (NCPD), Kenya National Bureau of Statistics, Council of Governors, County Governments.

RESEARCH ETHICS REVIEW & OVERSIGHT:

Amref Ethics and Scientific Review Committee, University of Nairobi, National Commission for Science, Technology and Innovation.

THEMATIC AREA 1:

HIV & SEXUALLY TRANSMITTED INFECTIONS FOR KEY POPULATIONS

Key populations (female sex workers, men having sex with men, transgender persons and people who inject drugs) in Kenya have disproportionately higher prevalence of HIV (29.3% among female sex workers, 18.2% among men having sex with men and 18.3% among PWID). The National AIDS and STD Control Program (NASCOP) recommends a balanced approach of biomedical, behavioural and structural interventions for HIV prevention and promoting SRH among KP in Kenya. Out of the estimated 133,000 FSW, 18,000 MSM and 4,305 TG in Kenya, there are about 10,000 FSW, 3,000 MSM and 435 TG in Mombasa County and 4,000 FSW in Kilifi County.



AFYA PWANI & STAWISHA PWANI KEY POPULATION PROGRAMS:

ICRHK implemented a KP program in Mombasa and Kilifi counties under the USAID-funded Afya Pwani sub-award between January 2021 and July 2021, and Stawisha Pwani sub-award between August 2021 and December 2021. Under Afya Pwani, ICRHK collaborated with Pathfinder International, the prime awardee for the USAID grant. Under Stawisha Pwani, ICRHK collaborated with LVCT Health. The KP program aimed to provide comprehensive HIV/AIDS prevention, care and treatment, and other SRH services to FSW, MSM and transgender persons through a peer-led demand creation approach with clinical services offered at specialized sex worker clinics (drop-in service centers) and through outreach.

ICRHK currently runs six drop-in centers. Mombasa County in Mvita, Likoni, & Kisauni & Kilifi County in Mtwapa, Kilifi and Malindi towns.

10,000

FSW in Mombasa County

3,000

MSM in Kilifi County

4,000

FSW in Kilifi County

MAIN ACHIEVEMENTS & THE RELEVANCE OF THESE ACHIEVEMENTS

Scaled up case identification strategies such as Assisted Partner Notification Services (aPNS), Expanded Peer Outreach Approach (EPOA) and Risk Network Referrals (RNR). A total of 517 FSW, 265 MSM and 59 TG newly identified HIV positive in Mombasa and Kilifi counties in 2021. The linkage to ART for HIV positive FSW was 95%, that among MSM was 100% and among TG was 67%. Apart from strengthening pre- and post-test counselling by building the capacity of the engaged service providers, ICRHK also relied on peer navigators to provide peer psychosocial support to newly identified positives to encourage them to start ART as well as to facilitate follow up for those not immediately linked. ICRHK engaged a total of 12 peer navigators in 2021.

Scaled up differentiated care as an adaptation to the COVID-19 pandemic by offering multi-month scripting (MMS) for stable HIV positive KP. A total of 1,160 FSW, 277 MSM and 47 TG offered MMS in Mombasa and Kilifi counties in 2021.

Disseminated program performance at county-level technical working groups for KP in Mombasa and Kilifi counties. TWGs make decisions on key elements of KP programming.

16,233

FSW in Mombasa & Kilifi counties reached with HIV prevention interventions at hotspots through peer educators. 67% of the sex workers reached were tested for HIV & received their results

2,431

MSM & 379 Transgender received health education, information materials, condoms and lubricants, in Mombasa County. 73% of the MSM received HIV testing services

1,101

FSW, 433 MSM & 94 Transgender persons initiated and continued oral PrEP at ICRHK-supported sites in 2021

1,305

FSW in Mombasa & Kilifi counties were screened for cervical cancer and those with suspicious lesions referred for further management



UNFPA-SUPPORTED FEMALE SEX WORKERS PROGRAM

The Kilifi County AIDS and STIs Control Officer coordinated the mapping of sex work hot spots in Mtwapa and Kilifi towns. Bars, nightclubs, casinos, strip clubs, hotels, parks, and beaches were among the hotspots. There were also a number of home-based and internet-based sex work venues and websites listed. With the help of the National AIDS and STI Program (NASCO) office, the program conducted a size estimation exercise to estimate the total sex worker population in these areas. During the mapping and size estimation, the project chose and engaged FSW who fit the profile of potential peer educators (PEs) based on national guidelines for implementing FSW programming.

A total of 38 peer educators were chosen. The peer educators participated in a one-week peer education training using the national peer education curriculum.

Peer educators provided health education to fellow sex workers at hotspots, distributed condoms, lubricants, and information materials, responded to violence, and connected peers to clinical services. Drop-in centers (DICs) and outreach provided comprehensive clinical services for FSW. HIV testing and counselling, antiretroviral therapy, including oral PrEP, STI treatment, cervical cancer screening and referral, family planning, psychosocial counselling, and GBV response and referral were among the services provided.



MAIN ACHIEVEMENTS AND THE RELEVANCE OF THESE ACHIEVEMENTS

4,700

FSW were reached with HIV prevention interventions at hotspots through peer educators. 52% of the sex workers reached were tested for HIV and received their results

213

FSW were diagnosed HIV positive and all were initiated on anti-retroviral therapy

428

FSW were initiated & continued oral PrEP

4,645

FSW were screened for STI with 198 diagnosed STI positive & offered treatment

1,178

FSW were screened for cervical cancer with 13 suspicious cases referred for further management

613

FSW received family planning for the first time

5

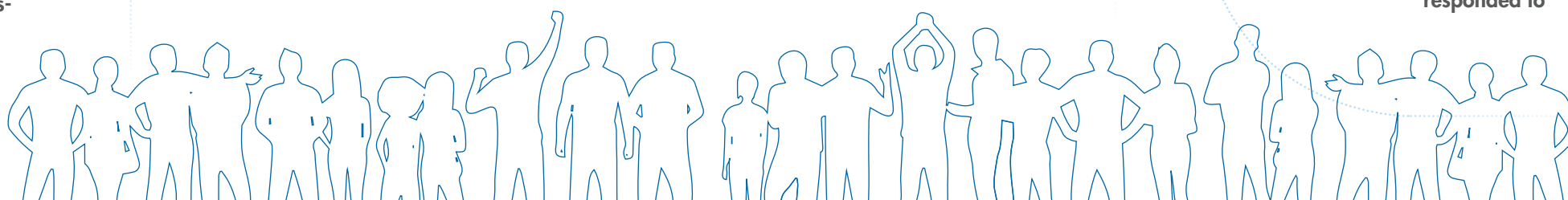
paralegals engaged

650

physical violence cases that were reported responded to

18

sexual violence cases that were reported responded to

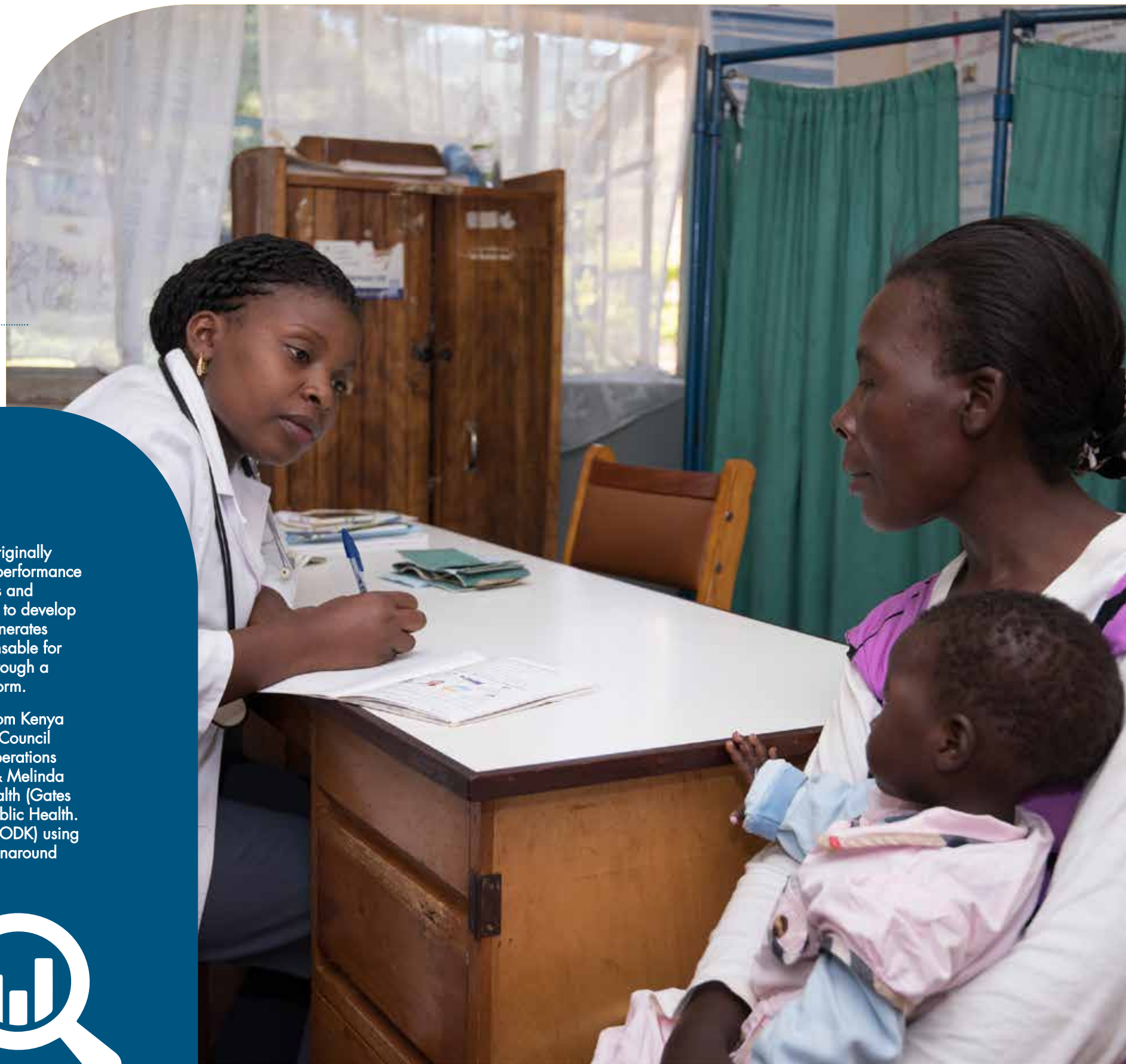


THEMATIC AREA 2: **FAMILY PLANNING, MATERNAL & CHILD HEALTH**

PERFORMANCE MONITORING FOR ACTION (PMA):

Performance Monitoring and Accountability was originally designed to respond to the need for more regular performance monitoring data to track progress against the goals and principles of Family Planning 2020. PMA's focus is to develop a transformational and actionable platform that generates and disseminates data, which prove to be indispensable for governments, donors, managers and advocates through a sustainable, innovation-driven data collection platform.

The implementation is led by MOH with support from Kenya National bureau of statistics (KNBS) and National Council of Population and Development (NCPD). Survey operations are led by ICRHK with technical support from Bill & Melinda Gates Institute for Population and Reproductive Health (Gates Institute) at Johns Hopkins' Bloomberg School of Public Health. PMA collects real-time data through open data kit (ODK) using mobile devices (mobile phones and/or tablets). Turnaround time for data collection, cleaning, analysis and final dissemination is 12 weeks.





“The authentic value of data is recognized by the government when it is used to generate and stimulate new ideas, discussions and informed action. PMA has now reached to that level; the data is appreciated and its being used widely to contribute to designing targeted FP interventions and it’s also contributing to policy review and implementation processes to respond to the needs of our communities.”

Dr. Stephen Kaliti, Head of Reproductive and Maternal Health Division, Ministry of Health



DATA COLLECTION:

On the 18th and 22nd of October 2021, we held Phase 3 training in Machakos Town. The goal of the refresher training was for field staff to review and understand the necessary data collection tools, additional survey modules, and the process of interviewing respondents at home and at service delivery points. In addition, trainees were guided through a number of scenarios that they would likely encounter during data collection.

A total of 263 enumerators were trained despite the challenges and hardships posed by the COVID-19 pandemic. Data collection began in the middle of November 2021. Women of reproductive age (15-49) from selected households in 11 Kenyan counties were interviewed. We enlisted the help of enumerators, the community, county administrative authorities, government agencies, and the ethical regulatory commission at both the national and global levels to carry out the activity. Joint data collection monitoring visits were conducted by staff with support from the Ministry of Health (MOH), the National Council for Population and Development (NCPD), and the Kenya National Bureau of Statistics (KNBS) to ensure the quality of data generated by field staff.



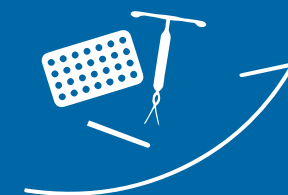
DISSEMINATION OF FINDINGS:

The project annually communicates survey findings to stakeholders at the national and subnational levels with the goal of informing policy and programs, with a particular focus on assisting decision makers in understanding what is working and what is not, allowing for more sensitive and responsive decisions to changing needs.

The results of the Phase 2 National Family Planning Survey were released in Nairobi by PMA in collaboration with the Ministry of Health (MOH), the National Council for Population and Development (NCPD), and the Kenya National Bureau of Statistics (KNBS). The increase in the use of modern contraceptives from 56 percent in 2010 to 61 percent in 2020 was a key finding, indicating progress toward the ICPD25 commitments.

ENGAGEMENT OF STAKEHOLDERS:

Actively engaged with the Ministry of Health (MOH), National Council for Population and Development (NCPD) and Kenya National Bureau of Statistics (KNBS) in several activities. We engaged MOH at the National Family Planning Dialogue Forum where PMA findings on quality of care was presented. We also joined the National Family Planning Technical Working Group in which the National Costed Implementation Plan was reviewed. The project team was part of the task force that participated in the review to ensure that PMA data contributed towards the development of the plan. KNBS and NCPD supported project activities including enumerator trainings, data collection, supportive supervision, data interrogation workshops, PAB meetings among others.



The increase in the use of modern contraceptives from 56% in 2010 to 61% in 2020 was a key finding



“In Kericho County PMA data has informed targeted activities that we have undertaken to increase demand. This has been realized through joint efforts between county and partners like The Challenge Initiative who have supported outreaches to the communities. We have also trained staff on long term methods and counselled clients on the method mix such that they are able to choose what they need within reach.”

**Dr. Betty Langat
County Director of Health, Kericho**



A NETWORK OF CARE TO IMPROVE THE CONTINUITY & QUALITY OF MATERNAL AND PERINATAL SERVICES IN MAKUENI COUNTY, KENYA

A Network of Care (NoC) for maternal and perinatal health is defined by the World Health Organization (WHO) as a group of public and/or private health facilities and health workers in a defined geographic area that are intentionally interconnected to promote multi-disciplinary teamwork and collaborative learning. A functional NoC results in collaborative and coordinated continuity of high-quality, respectful maternal and perinatal care, allowing health systems to be more efficient and resilient.

Providers in outlying facilities may lack updated guidelines for managing complications, pre-referral care, or clear indications for referral; or they may lack the skills, confidence, authority, and/or supplies necessary to provide high-quality care or make referral decisions. When emergency cases arrive, referral facilities may not have enough staff, skilled providers, equipment/supplies, or appropriate guidelines to handle them. The private sector rarely communicates or coordinates with the public health system, which is a missed opportunity to maximize all available resources to reduce maternal and perinatal morbidity and mortality. New approaches are required to create more efficient and effective systems for providing high-quality, person-centered care at each point of care in the public and private sectors, as well as to ensure coordinated, and continuous care throughout the continuum of care.

This study aims to elucidate key elements of the NoC that may help contribute to improvements in the continuity and quality of MNH care and outcomes in Makueni County, Kenya, by applying the NoC framework and studying the acceptability and feasibility of establishing a NoC.



60

**health facilities
purposely selected
were included**

STUDY METHODOLOGY:

The study was mixed methods implementation research conducted in Makueni County.

60 health facilities purposively selected were included; these were public, private, and faith based facilities with a representation of level two, three, four and five facilities.

Quantitative and qualitative data was collected at baseline, and next steps are data collection; quarterly (for quantitative data), at the study's midpoint, and at the study's conclusion (for qualitative).

For qualitative, the study used Key Informant Interviews (KIs) and Focus Group Discussions (FGDs) to explore views from the health workers on collaboration amongst health facilities in Makueni County and Makueni

sub-counties' facilities, particularly related to maternal and neonatal care, referrals, as well as to understand the feasibility and acceptability of establishing a Network of Care (NoC) intervention in the county.

A total of 24 KIs and 7 FGDs were conducted at baseline. For quantitative; a USSD phone survey was launched to selected health workers involved in MNH referrals at the 60 health facilities.

24

**KIs & 7 FGDs were
conducted at baseline**

THEMATIC AREA 3:

SEXUAL & GENDER-BASED VIOLENCE

THE AMPLIFY CHANGE PROJECT

Over two-thirds of Kenyan girls are estimated to have experienced sexual or intimate partner violence at some point in their lives. The Amplify Change project intensifies child protection interventions while raising awareness on the suffering of other vulnerable groups. By effectively supporting those who are best positioned to lead change and amplifying their voices, this approach will forge long-term partnerships/relationships toward a common goal of building stronger and more inclusive movements for sexual reproductive health rights.



APPROACH:

The project works with Area Advisory Councils to strengthen grass-roots SGBV coordination, particularly in the area of child protection. We do this in collaboration with other stakeholders who share our values. Through evidence generation and up scaling of counseling, legal, and clinical management services, the project aims to improve post-rape care service delivery at the Gender-Based Violence and Recovery Centre Coast General Teaching and Referral Hospital.



ACHIEVEMENTS:

The project helped to re-establish the Changamwe AAC, which now has 23 members, while also providing training on SGBV best practices, particularly in the area of child protection. Because it is no longer part of the Jomvu Sub-County, this Council has been inactive since 2019. It has also educated 22 members of the Kisauni AAC about the importance of incorporating SGBV into their activities, making it a top priority in the region's child protection agenda. It also provides ongoing technical assistance to the two Councils' strategic plans.

ICRH-K has also secured a partnership with the Technical University of Mombasa to produce a student-led podcast aimed at raising awareness about sexual and gender-based violence. The University's position in academia and the broader community will help the podcast and its message gain traction.

The GBVRC at the Coast General Teaching and Referral Hospital received minor

renovations in order to improve its service delivery capabilities.

To better serve the 88 percent of children who visit the Centre, a dedicated child therapy room has been established. It was launched on December 2, 2021, as the first of its kind in Mombasa County. This initiative was also a step toward improving the relationship between ICRH-K and CGTRH in order to provide better service.

Following that, the project helped to fund the hiring of a child therapist who will provide psychosocial support to children.

As a complement to traditional in-person counseling, the Centre has begun offering tele-counseling services.

The survivors' completion rate of five counseling sessions will hopefully be improved as a result of this. Since then, the Children's Department and the Child-Line agency, among others, have referred more patients to the Centre.

THE SEXUAL VIOLENCE MEASUREMENT STUDY

The Equality Effect partnered with ICRH-K to conduct a baseline study to determine the prevalence of sexual violence among school-aged children and adolescents aged 12 to 21 years in four counties representing Kenya's Coastal and Western regions: Kilifi, Homabay, Kisumu, and Kwale. The study will be used as a benchmark for future Equality Effect interventions, such as training police officers and officers at gender desks on best practices for dealing with sexual violence survivors.

The study looked at both female and male students to see if they had previously been defiled or if they knew of another child who had been defiled. It also wanted to know how they felt about how adults and police officers treated defilement cases in the community as a predictor of their willingness to report defilement cases. The intended sample size was 1200 people.

The schools were chosen because they were within a 10-kilometer radius of a police station or post, ensuring proximity and relevance for subsequent intervention. The baseline cross-sectional survey was conducted between June & August 2021, and the study used a descriptive research design with surveys.

FINDINGS:

There was a 3 days intensive training of 12 research assistants (4 per County) on ethical research and sexual violence. 1204 responses were obtained across the four counties thus meeting the intended target. The interviews were also successfully conducted at a majority of the schools. Majority of the school administrations were impressed

with the study and acknowledged the need for further intervention on matters sexual violence within their Counties. A preliminary report has been developed with a detailed analysis. A protocol paper/manuscript on the Study experience is ongoing.



Conduct a baseline study to determine the prevalence of sexual violence among school-aged children & adolescents aged 12 - 21 years in 4 counties



Baseline cross-sectional survey was conducted between Jun & Aug 2021

1200

people - intended sample size

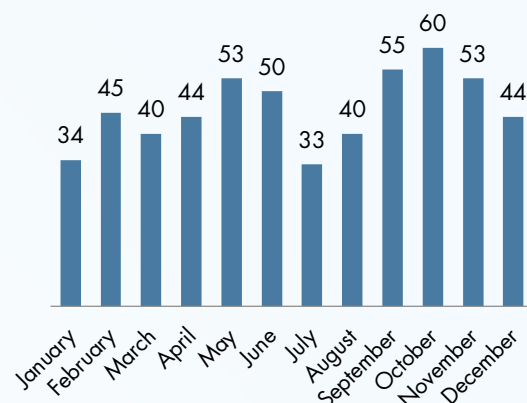
THE SEXUAL & GENDER-BASED VIOLENCE CENTRE

The GBVRC (Sexual and Gender-Based Violence Recovery Center) was established in 2007 as a model SGBV Center through a collaboration between ICRHK, Coast General Hospital, and Mombasa Municipality (now Mombasa County). SGBV services are provided alongside other services at a public health facility's outpatient department in this model. It was hoped that by using this model, populations who do not routinely report sexual violence would find it easier to do so. Men and children are among them. This report covers activities related to support for the Gender Based Violence Recovery Center (GBVRC) at the Coast General Hospital that took place in 2021.

SGBV SERVICES:

The CGT and RH-GBVRC continued to provide medical, psychosocial and referral support to survivors of GBV who were presented at the hospital. Since the opening of the Center in May 2007 to December 2021, a total of 8,971 survivors have been attended to. 551 survivors were attended to during the year 2021.

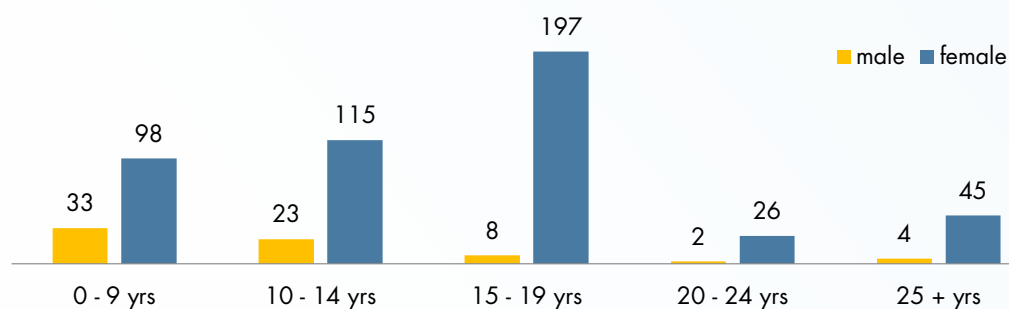
The graph below shows number of survivors per month



The table below shows the number of survivors per assault type distribution. The most common type of sexual violence type was defilement/rape as shown in the table below.

Type of assault	Male	Female
Attempted rape/ defilement	2	19
Sexual assault	3	24
Defilement/ rape	65	438
Total	70	481

The graph below shows the number of survivors seen by age distribution



Out of the **551** survivors who received care:

207

survivors; 38 males & 169 females came within 72 hours and were offered PEP

11

male perpetrators were brought to the center under police custody for collection of samples.

1

teenage girl aged 16 years became HIV infected and was referred to Shimo la Tewa Comprehensive Care Centre (CCC) for care & treatment

59

girls survivors aged 10 -17 years came in pregnant as a result of defilement

24

had given birth

LEGAL FOLLOW-UP:

To follow up on these cases, the ICRHK hired 20 paralegals. As part of the legal process, all of the cases were followed up on at the police station and in the courts. In addition, the organization worked with legal structures to strengthen the legal response to sexual violence through area court users committees. In total, 76 cases were resolved, with the perpetrators facing sentences ranging from five years to life in prison.

Table below shows outcome of referred cases at the different stations.

Station	Cases under police investigation	Cases pending before court	Jugement concluded
Nyali/Kongowea	18	24	16 cases concluded
Mjambere	2	26	3 cases withdrawn as accused went missing
Kiembeni	8	39	8 cases concluded
Kadzandani	2	16	0
Bamburi	4	19	9
Junda	6	22	0
Makupa	19	29	0
Chaani	2	21	2 cases concluded
Mikindani/Jomvu	0	46	7 cases concluded
Changamwe	0	48	29 cases concluded
Mbungoni	3	6	0
Likoni	0	4	0
Central Police	6	14	1 case concluded
Child protection Unit	5	4	1 case concluded
TOTAL	75	318	76 cases concluded

CHILD THERAPY

The child therapy play room was inaugurated on 2nd December 2021 with funding from the Ampify Change Network. A total of 19 survivors have been counselled through play therapy. Major psychological issues identified included fear, anxiety, regression and reintegration.

19



survivors have been counselled through play therapy

THE 160 GIRLS VIRTUAL JUSTICE CLUBS

The “160 Girls” project is a legal advocacy initiative that aims to achieve justice for child rape victims, and protection against rape for all children in Kenya. The goal of the project is to enforce existing laws in Kenya to protect girls from sexual violence. This is based on a 2013 landmark ruling where on May 27, 2013, the High Court of Kenya ruled that the police treatment of the “160 Girls” petitioners’ claims violated their human rights, and that the police treatment of defilement had created a climate of impunity for defilement, which rendered them indirectly responsible for the harms inflicted by the perpetrators. This project strives to equip children (girls and boys) with requisite knowledge on SV for a better understanding of their rights, and enhance community awareness all while leveraging on technology.



THE APPROACH:

Awareness creation among children on their rights where defilement is concerned and the roles of the different stakeholders (parents, police, teachers and community).

Mobilizing participation among children in hotspot counties (Kwale and Kilifi) through engagement with schools and general community.

Establishment of virtual justice clubs in the hotspot regions. Children will be educated using tablets.

Enforcing collaborative efforts between different stakeholders including teachers, parents, police and the general community.

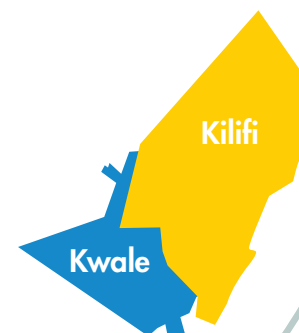
“I need to know what to do if or when I am violated... I too can fight for my right and for the rights of other children like me”

Anonymous

“Thank you for remembering the boy-child in this project. So that we can also be protection and encouraged to speak out”

Anonymous

OUTCOME & POTENTIAL IMPACT:



Successful device training & sexual violence sensitization of various direct beneficiaries as below:

Children	Parents	Teachers	Police	Total
226	221	9	3	459

Children	Parents	Teachers	Police	Total
225	225	9	3	462

Total	451	446	18	6	921
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THEMATIC AREA 4:

ADOLESCENT & YOUNG PEOPLE SEXUAL & REPRODUCTIVE HEALTH & RIGHTS

LINDA MTOTO PROJECT (PREVENTING COMMERCIAL SEXUAL EXPLOITATION OF CHILDREN)

Commercial sexual exploitation of children is a worldwide issue, affecting an estimated 1.8 million children. In many parts of Africa, including Kenya, CSEC is common. Approximately 30,000 children in Kenya are affected. CSE has physical, social, & psychological consequences for children. The Linda Mtoto project aims to improve the health of children at risk of or affected by CSEC in Kisauni and Jomvu Sub-County by identifying & supporting community structures.



OUR APPROACH:

Activities in the Linda Mtoto project are categorized into three main components:

The first component seeks to identify Children survivors of CSE through Community Health Volunteers.

In 2021, 450 children were identified, bringing the total number of children identified by December 2021 to 708 (630F:70M). The target number for the entire project is 885 children.

88% of the children identified were female while 11% were male. The median age of the children was 15 years

The most common type of CSEC was survival sex, followed by tourism sex and early marriage.

All of the children who were enrolled in the project were required to receive a standard comprehensive care package that included psychosocial support, medical management, and referral to a support group. Six group counseling sessions were provided to 304 children. Project counsellors used the PHQ9 questionnaire to assess depression and anxiety trends among the children at each time point. Longitudinal analysis revealed that as children progressed through their counseling sessions, their depression levels decreased.

The school intervention centered on teaching life skills and child protection to in-school peer educators. In 2021, a total of 200 peer educators will have received the same training. The peer educators were tasked with educating their peers in the school setting.

Institutional ties were strengthened by assisting children who required vocational or educational assistance. In 2021, the project provided educational and vocational sup-

port to a total of 20 children. 105 caregivers/parents (100M: 5F) were trained in financial/resource management and then assigned to saving groups. Participants were also taught value-added and soft skills such as how to make charcoal briquettes, soap, and agribusiness. The goal of this intervention was to increase household income and thus reduce family poverty. In addition, 155 caregivers (145F:10M) received positive parenting training.

Schoolchildren competed in an inter-school essay and drawing competition in November 2021. The competition's theme was 'child rights.' On the 4th of December 2021, over 400 submissions were received, and winners were an-

nounced at a prize giving ceremony held at Mtopanga Primary School. School speak out boxes and murals were also unveiled at the event.



ADDITIONAL ACHIEVEMENTS & THE RELEVANCE OF THESE ACHIEVEMENTS

Formation of 4 registered self-help groups after completing the finance/resource management training. The groups promote resilience and create a support network amongst caregivers/parents of children survivors of CSE. The groups are also eligible to receiving funds and loans from Government and Non-Government financing institutions.

20

children supported with educational/vocational support in 2021



128

child beneficiaries received support in form of legal aid, educational/vocational from the Children's department & other Non-Government Organizations

Linda Mtoto Inter-School Competition and subsequently the prize giving ceremony. This activity strengthened existing partnerships and enhanced visibility of the organization in the Government and Private sector.

A total of 128 child beneficiaries received support in form of legal aid, educational/vocational from the Children's department and other Non-Government Organizations. The project supported 20 children with educational/vocational support in 2021.

A total of 250 caregivers/parents trained on positive parenting out of the initial target of 200 caregivers/parents.

THEMATIC AREA 5:

HEALTH MONITORING & EVALUATION

THIRD-PARTY MONITORING FOR DESIP

In 2020, ICRHK, Hera and Aga Khan University (AKU) were engaged by the FCDO to conduct third-party monitoring and process evaluation to FCDO's largest FP funded program in Kenya, the Delivering Sustainable and Equitable Increases in Family Planning (DESIP). DESIP was funded by FCDO for four years and aims to scale-up FP among marginalized communities in 19 counties in Kenya. The third-party monitoring team would provide FCDO with independent, verifiable data on the progress of DESIP in achieving its objectives as outlined in the logical framework.



METHODOLOGY:

The DESIP implementing partners provided a list of all DESIP facilities. A multistage sampling strategy was used, with at least one facility selected in each ward of each county visited. This ensured that each of the DESIP counties was fully covered. Furthermore, a mix of facilities was chosen based on ownership in the counties visited in each round. In addition, a list of private pharmacies was obtained from the implementation partners for the social marketing component, and a random selection was made.

Three methods of data collection were used: health facility assessments (HFAs), key informant interviews (KIIs), and focus group discussions (FGDs). The KIIs were given to county reproductive health coordinators, county pharmacies, health facility in-charges, and private pharmacy owners. FGDs were conducted with women of reproductive age (WRA), men, and adolescents aged 15 to 19 years.

ACHIEVEMENTS:

Two rounds of primary data collection were conducted as follows:

In January 2021, 68 facilities (8 faith-based, 18 private, and 45 public facilities) in Baringo, Homabay, Migori, and Narok were visited to conduct health facility assessments using a structured questionnaire which included sections on family planning (FP) services and provision and data verification. The period covered was October to December 2020. Further, 94 key informant interviews were conducted in the 4 counties, 68 facilities, and 22 pharmacies. To collect views on the availability of commodities, access to FP, barriers, counseling, and preferred methods from FP beneficiaries, a total of eight (8) FGDs were conducted in the four counties. Each focus group consisted of 12 participants including 6 separate groups with women of reproductive age (WRA), adolescent girls 15 to 19 years, and with men.

In April 2021, the second round of primary data collection was conducted covering January to March 2021 implementation period. A total of 75 facilities were visited in Elgeyo Marakwet, West Pokot, and Samburu counties. Health facility assessment (HFA) was conducted in all the facilities. In total, 6 FGDs were conducted; 2 for WRA, 3 for adolescent girls, and 1 for men. Additionally, we collected information about social marketing products in private pharmacies by conducting a total of 16 KIIs. The second component included extracting services and commodities secondary data from KHIS aggregate and calculating the couple of year of protection (CYP) and stock levels in all the DESIP counties.



THIRD-PARTY MONITORING FOR THE IN THEIR HANDS (ITH) PROJECT

ITH is implemented by Triggerise with financing from Development Impact Bond (DIB) issued by the Children's Investment Fund Foundation (Investor) to Foreign, Commonwealth & Development Office (Outcome payer). It generates demand for contraceptive and SRH services among adolescent girls who obtain information from a variety of sources including peers, mobilizers, and online campaigns.

The ITH evaluation is conducted by ICRHK in collaboration with Hera to provide independent monitoring of the project throughout its implementation as follows:

A baseline and end-line survey of adolescent girls (15-19 years) modern contraceptive prevalence (mCPR) in ITH project areas in 16 counties.

Two rounds of data verification by comparing raw data from the Tiko system database to reports submitted to CIFF by Triggerise.

Two rounds of field data verification, verifying services reported by providers to Triggerise against registers at service delivery points.



APPROACH:

Triggerise provided a list of facilities that provide services via the Tiko system. A high-volume health facility was identified, as well as the ward in which the facility is located. The KNBS was hired to draw enumeration areas (EAs) based on the 2019 census using a master sampling frame. Using high-volume service facilities, EAs were drawn within a 5-kilometer radius. Geo-coordinates were used to locate the facility. The KNBS selected 354 EAs with a total of 37,756 households. There were two stages to the survey. A resident enumerator first listed all the households in an EA, identifying the number of adolescent girls in the household and those who have ever been sexually

active. After informed consent was obtained from them or their guardians, all those who were confirmed to have ever been sexually active were interviewed in the second stage.



ACHIEVEMENTS:

Two rounds of field data verification conducted:

50 private clinics & pharmacies visited to verify family planning services provided in three months to determine the quality of data reported to the funding agency.

Baseline household-level survey conducted in **June & July in 16 counties**.

The following are the findings from the survey:

39,685

households were identified

15,706

adolescent girls were identified

354

enumeration areas were identified

5,399

confirmed that they had ever been sexually active

5,274

never had sex

5,033

were not available at the time of the visit or did not respond

5,399

girls who had confirmed to ever have been sexually active were included in the sample to be interviewed

4,953

among them were interviewed, representing 35% of all adolescent girls in the sampling area

4,850

sample size was reached

25.3%

The overall weighted mCPR



The results of the first population-based household survey were compiled into a report and submitted to the funding agency with a copy to the implementing partner.

THIRD-PARTY MONITORING FOR DESIP

In 2020, ICRHK, Hera and Aga Khan University (AKU) were engaged by the FCDO to conduct third-party monitoring and process evaluation to FCDO's largest FP funded program in Kenya, the Delivering Sustainable & Equitable Increases in Family Planning (DESIP). DESIP was funded by FCDO for four years and aims to scale-up FP among marginalized communities in 19 counties in Kenya.

The third-party monitoring team would provide FCDO with independent, verifiable data on the progress of DESIP in achieving its objectives as outlined in the logical framework.

4

years duration



Scale-up FP among marginalized communities

19

counties in Kenya



CHALLENGES DURING HOUSEHOLD-LEVEL SURVEY & HOW THEY WERE MITIGATED

COVID-19 related limitations: There was an increase in covid-19 infections in some study areas (Kisumu, Kakamega, Busia, Homabay and Migori). As a result, a curfew from 7 PM to 4 AM was introduced which limited the time the REs could conduct interviews. This was mitigated by conducting some interviews during the weekends and prolonging the survey duration

Adolescent girls in school: The majority of adolescents aged 15 to 19, and especially among the younger age groups, were enrolled in school, including some in boarding schools. Students attending day schools were only available after 5 PM and on weekends. The resident

enumerators (REs) therefore visited most homes around 5 PM and on weekends. Additionally, they made multiple visits to the homes of adolescent girls if those sampled were not present. This also increased the duration of the survey and the overall cost of the survey

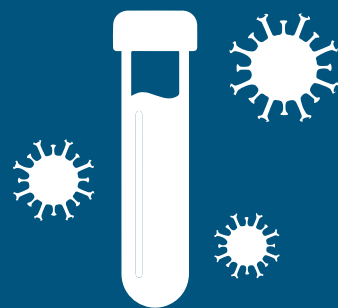
Insecurity: This was reported in a few enumeration areas, most notably those located in Nairobi and Mombasa's urban slums. REs reported being unable to access some households in a Nairobi slum area due to insecurity. The team consulted with the KNBS and a list of additional households was provided to replace the inaccessible households

THEMATIC AREA 6:

COVID-19 RELATED RESEARCH

VALUES & PREFERENCES FOR COVID-19 SELF-TESTING IN LOW & MIDDLE-INCOME COUNTRIES; A STUDY IN KENYA

Self-testing for COVID-19 holds the potential to increase access to COVID-19 testing. World-wide there are three modes of testing for COVID 19, Real-Time PCR (GOLD STANDARD), lateral assays and self-testing (ST). Although self-testing is available in some parts of the world, WHO guidelines on ST are lacking. In Kenya, ST is yet to be approved for use and no applications for licensing for these devices are available. We conducted a study to determine the values and preferences for COVID-19 self-testing in Kenya.



METHODOLOGY:

This was a mixed-methods study in two counties in Kenya; Mombasa serving as urban site, while Taita Taveta County a rural site. Data was collected from four population groups: general population members, health care workers, general population representatives, and potential COVID-19 self-testing implementers. Quantitative data was collected on an android-device and real-time uploaded to REDCap, a cloud-based server. Health workers online survey was also conducted. Qualitative interviews were in form of focused group discussions and in-depth interviews. A total of

36 interviews (30 in-depth interviews and six focus group discussions) were conducted.

A household survey had 392 general participants and online survey with 483 health workers.



MAIN ACHIEVEMENTS & THE RELEVANCE OF THESE ACHIEVEMENTS

The findings from the study have been included as part of the Interim guidance on COVID 19 self-testing by World Health Organization (WHO). "Use of SARS-CoV-2 antigen-detection rapid diagnostic tests for COVID-19 self-testing".

Findings from qualitative data presented during the virtual meeting for 2021 Annual meeting for African Studies association (ASA).

Two manuscripts: One from the quantitative data and the other on qualitative findings respectively have been submitted to journals.

2

manuscripts submitted to journals



Qualitative data presented at the 2021 Annual meeting for African Studies association (ASA)

NOTABLE EVENTS & ACHIEVEMENTS IN 2021



PUBLICATIONS

Manguro G.O, Mwaisaka J, Okoro D, Korir K, Owira P, Githinji G, Olajide A, Temmerman M. *Failing the Rights: Sexual Vulnerability, Access to Services and Barriers to Contraceptives among Adolescents in Narok County, Kenya*, International Journal of Human Rights in Healthcare; 2021 August 23

Gichangi P, Waithaka M, Thiongo M, Agwanda A, Radloff S, Tsui A, Zimmerman L, Temmerman M. *Demand satisfied by modern contraceptive among married women of reproductive age in Kenya*. PLoS One. 2021 Apr 9

Mwaisaka J, Gonsalves L, Thiongo M, Waithaka M, Sidha H, Alfred O, Mukiira C, Gichangi P. *Young People's Experiences Using an On-Demand Mobile Health Sexual and Reproductive Health Text Message Intervention in Kenya: Qualitative Study*. JMIR Mhealth Uhealth. 2021 Jan 15

Thiongo MN, Gichangi PB, Waithaka M, Tsui A, Zimmerman LA, Radloff S, Temmerman M, Ahmed S. *Missed opportunities for family planning counselling among postpartum women in eleven counties in Kenya*. BMC Public Health. 2022 Feb 8;22(1):253. doi: 10.1186/s12889-022-12623-0. PMID: 35135514.

Griffins Manguro, Abednego Musau, Daniel Were, Soud Tengah, Brian Wakhutu, Jason Reed, Marya Plotkin, Stanley Luchters, Peter Gichangi, Marleen Temmerman, *Increased condom use among Key Populations using oral PrEP in Kenya: results from large scale programmatic surveillance*; BMC Public Health ; 2022 February

Gichangi P, Gonsalves L, Mwaisaka J, Thiongo M, Habib N, Waithaka M, Tamrat T, Agwanda A, Sidha H, Temmerman M, Say L. *Busting contraception myths and misconceptions among youth in Kwale County, Kenya: results of a digital health randomised control trial*. BMJ Open. 2022 Jan 6;12(1):e047426. doi: 10.1136/bmjopen-2020-047426. PMID: 34992099; PMCID: PMC8739061.

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